

Document info

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MRI Abdomen WO/W Contrast

Patient:	KAREN CLAYBORNE	DOB:	Jan 11, 1966
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REPORT

MRI Abdomen WO/W Contrast

CLINICAL HISTORY: Female, 59 years old, with history of Lung cancer, uncinate process mass.

TECHNIQUE: Routine multiplanar, multisequence imaging was performed of the abdomen with contrast.

COMPARISON EXAMS: CT from April 2, 2025

FINDINGS: Detail is limited by motion on multiple sequences. There is a moderate to large right pleural effusion. Left lung base appears clear. There is a nonenhancing 1.5 cm high T2 signal cystic within the liver adjacent the porta hepatis. There is a subtle nonspecific 10 mm area of intermediate T2 signal involving the right hepatic lobe, segment 7, which demonstrates homogenous arterial enhancement which persists, nonspecific. There are additional subcentimeter high T2 signal foci of increased T2 signal, too small to characterize without definitive associated visualized enhancement. The spleen, adrenal glands and visualized pancreas is unremarkable. A definitive pancreatic lesion is not seen although sensitivity is limited by motion. The gallbladder is contracted. No biliary ductal dilatation is evident. There is a subcentimeter high T2 signal focus along the lower pole the right kidney without enhancement. Cyst is

suspected. Additional smaller foci are also seen without definitive abnormal renal enhancement or hydronephrosis. Aorta is normal caliber. Small bowel loops are normal caliber. Prominent stool seen within the colon. There is an enlarged nodular uterus suspicious for fibroid measuring up to 8 cm in size. No ascites is identified.

There is increased T2 signal within the distal cord suspicious for syrinx which could be further evaluated with dedicated MRI imaging of the thoracic spine as appropriate clinically. There is mild superior height loss involving the L3 vertebral body. This demonstrates focal nodular enhancement. Nonacute etiology with secondary arthritic changes and enhancing Schmorl node is suspected. Correlation for point tenderness is recommended. There is additional height loss involving the T9 vertebral body with corresponding subtle enhancement suspicious for acute/subacute compression deformity. Detail is limited. This is better seen on prior CT and likewise be further assessed with dedicated MRI imaging of the thoracic spine as appropriate clinical.

IMPRESSION:

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1. Moderate to large right pleural effusion.
2. Nonspecific 10 mm area of intermediate T2 signal involving the right hepatic lobe, segment 7, which demonstrates homogenous arterial enhancement which persists, nonspecific. Consider CT PET scan dated exclude pathologic uptake as appropriate clinically. Alternatively consider conservative follow-up.
3. Additional subcentimeter high T2 signal foci within the liver, too small to characterize without definitive associated enhancement.
4. Enlarged nodular uterus suspicious for fibroid measuring up to 8 cm in size.
5. Increased T2 signal within the distal cord suspicious for syrinx. This could be further evaluated with dedicated MRI imaging of the thoracic spine as appropriate clinically.
6. Mild superior height loss involving the L3 vertebral body with focal nodular enhancement. Nonacute etiology with secondary arthritic changes and enhancing Schmorl node is suspected. Correlation for point tenderness is recommended.
7. Additional height loss involving the T9 vertebral body with corresponding subtle enhancement suspicious for acute/subacute compression deformity. Detail is limited. This is better seen on prior CT and likewise be further assessed with dedicated MRI imaging of the thoracic spine as appropriate clinically.
8. Unremarkable visualized pancreas. Detail is limited by motion.